

Patients do better at home

An orthopedic surgeon who has sat on both sides of the table — private practice, the C-suite, and back in the operating room — on what 90-day episodes will really ask of surgical care.



Dr. Scott Cooper

Orthopedic surgeon, Mercy
Former regional physician executive



Eddie Qureshi

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Thirty years in, and the measure that matters is the newest one

Dr. Scott Cooper spent the first third of his career as an owner in a private orthopedic group in Northwest Arkansas. In 2014 six of them integrated with Mercy. He went on to chair the clinic board, then serve a year as clinic president over more than 300 physicians and advanced practitioners, then two years as a regional physician executive — and then, with no regret, back to full-time orthopedics to finish his career where he started it.

That route gives him an uncommon read on what is coming. He has seen the constraints from behind the curtain, and he still has his hands in the case. Asked what actually signals quality after all this time, his answer is not the x-ray or the alignment. It is the patient-reported outcome — a measure the field largely ignored until the last five or ten years.

This guide distills his conversation with Eddie Qureshi on the Rainfall Health Podcast. It is written for surgeons, service-line leaders, and hospital executives preparing for TEAM and CJR-X — and for anyone weighing whether a 90-day episode is a threat or an opening.

“We're not here to create pretty x-rays and do elegant operations. The bottom line is: how are the patients doing?”

Dr. Scott Cooper

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FEATURED VOICES

Dr. Scott Cooper

Board-certified orthopedic surgeon at Mercy in Northwest Arkansas and a fellow of the American Academy of Orthopaedic Surgeons. Thirty years in practice, spanning private-practice ownership, a year as clinic president of a 300-plus clinician group, and two years as a regional physician executive.

Eddie Qureshi

Founder and CEO of Rainfall Health and host of the Rainfall Health Podcast. Also from Arkansas — a fact that comes up more than once.

Owner, board chair, president, executive, surgeon again

He calls the route circuitous, then concedes it isn't, really. Private practice at Ozark Orthopedics for the first third of a thirty-year career. In 2014, six surgeons in Benton County integrated with Mercy. Clinic board, then chair. When a president who had held the role for seventeen years stepped down, Cooper put his name in the hat and got a year leading more than 300 physicians and advanced practitioners. He then spent two years as a regional physician executive. He is now back in the operating room full time to finish his career — which he describes, without irony, as poetic. It is what he came to do.

2014 Integration Six private-practice surgeons join Sisters of Mercy Health System.	1 year Clinic president Leading a clinician group of well over 300 physicians and APPs.	2 years Physician executive A regional seat, and a front-row view of population health at scale.	Now Back in the OR A ramping surgical practice inside a growing group of young surgeons.
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What the two sides feel like from the inside

Private practice

A soft spot that has never gone away. He enjoyed being a small businessman and an owner, one of two or three managing partners with a full-time administrator beside them. He says a part of his heart will always be there.

Inside a system

Honestly bigger opportunities, and a surprise: he is an employee and has never felt like he had a boss. Keep your head down, practice hard, do well, treat people right.

He offers the leadership years the same way — briefer and different than he envisioned, and not regretted for a second, because they left him with a perspective most of his colleagues don't have. Seeing it from the side of the suits, as he puts it, is what made the bigger picture legible.

Go in with strong opinions and an open mind

Eddie's standing advice to clinicians is that if you're not at the table, you're on the menu. Cooper agrees on the encouragement and adds a caution about the posture. Physicians inclined toward leadership sometimes arrive with a bit of aggression — and the thing he'd tell them is to walk in prepared to see the organization the way its business leaders see it.

“The way you get things done is with cooperation, rather than butting your head up against the wall constantly.”

Dr. Scott Cooper

What doesn't change

Physicians can and should be strong in their opinions, and stand up for what's right — primarily for patients, and also for the physicians. That part is non-negotiable in his telling.

What you find behind the curtain

You go in planning to shake things up. Then you see the constraints — things in place that may or may not be changeable. You keep working for them. You change how you work for them.

The same goal, from different seats

Eddie's observation from hundreds of these conversations: clinical executives and administrative leaders almost always share the end goal, and differ only in vantage point. Nonprofit or for-profit, a US health system has to be financially sustainable to serve its community over time. Cooper's framing of value-based care is the shortest version anyone has offered on the show — do what's right by your patients, and build the frameworks around that.

BRING

The perspective of the people whose hands are in the case.

EXPECT

Constraints you didn't know existed, and a slower clock than the OR runs on.

GAIN

A view of the whole that makes you more useful to your colleagues and your patients.

Twenty years of measuring the wrong things

Early attempts at value-based care left a lot to be desired, and he says that without surprise — things should get better as lessons are learned. His example, names withheld, is the set of quality measures some Arkansas payers instituted twenty years ago. They may have been easy to report and easy to recover from data. They were not good proxies for the quality of care.

“We can talk about how pretty our hip replacement looks or how well aligned our knee replacement is. The patient doesn't care about that. How's the patient doing?”

Dr. Scott Cooper, on patient-reported outcomes

Still critically important

Infection rates. ED visits. Complications. All the typical measurables — he is not arguing they should go away, and elegant operations do correlate with happy patients.

But the bottom line

Patient-reported outcome measures, emphasized only in the last five or ten years. Very recent, and in his word, welcome. Ignored for far too long.

Under TEAM and CJR-X the point becomes structural rather than philosophical: patient experience and reported outcomes are wired into reimbursement. Eddie's question was whether tying dollars to the patient's own account of their recovery will actually move quality. Cooper thinks in a lot of ways it will — with one large caveat, which is the subject of the next chapter.

Tripling the window triples the variability

This is the part that worries him. Moving from TEAM's 30-day episode to CJR-X's 90-day episode introduces a great deal of variability, and the details of how it is defined matter enormously. Typically these models measure all-cause readmissions — and that is what gives physicians a headache when the accountability lands on them.

TEAM

30

days of episode accountability

Close enough to the surgery that most of what happens is plausibly connected to it. Includes hip and femur fracture episodes.

CJR-X

90

days of episode accountability

Long enough that life intervenes. Every acute care hospital in the country is in — roughly 3,300 of them.

His example: you replace a hip, and two and a half months later the patient has a car wreck and breaks an ankle. A lot of the time that lands inside the episode.

He hopes those definitions get fine-tuned — and he is clear that the fix is in the details of attribution, not in the idea of episodes. The same conversation, by his account, is more than twenty years old in Arkansas, and the measures have gotten better every cycle. Not perfect yet.

Where he looks instead: the efficiency nobody has taken yet

Rather than litigate the window, he spent the run-up to the conversation asking a different question — where are the real opportunities? He has one answer he keeps returning to, and it starts with a hip fracture in Little Rock.

His father went home the day after a hip fracture

Eighteen months ago his dad, in Little Rock, had an intertrochanteric fracture. A friend of Cooper's did the surgery, it went perfectly, and he went home the next day. Cooper is the first to list the advantages: a son who is an orthopedic surgeon, a brother and sister nearby, a competent wife at home, and direct access to the operating surgeon. Not every 87-year-old has that. But the case proves the thing can be done.

Meanwhile, the elective side already moved

At Mercy in Northwest Arkansas, a huge majority of primary hip and knee replacement patients go home the same day. Not outpatient extended stay — same-day surgery. Quality stayed high. Patient satisfaction stayed very high.

COVID accelerated a trend already underway. Patients didn't want to be in the hospital, there were no beds for them anyway, and the operating rooms were open. So the cases went ahead on the condition that nobody stayed the night.

“Ninety percent of preventing DVT after elective joint replacement is throwing you out of here today.”

Normal-risk patients now take a baby aspirin twice a day rather than the strong blood thinners of a decade ago. Getting up, going to the bathroom, going to the kitchen — that is most of the prophylaxis.

The difference is preparation. Elective patients are coached up for weeks — mentally, psychologically, they are ready for it. Nobody coaches up a fall. His question for the field is how to bend that curve anyway, given that the mindset is the constraint rather than the physiology.

The gap he wants closed

Hip fracture patients stay in the hospital as long as they need to in order to qualify for post-acute inpatient care. That is the norm around the country, and it is a norm, not a clinical necessity.

The clinical argument

At home, patients have to do for themselves. In a skilled nursing facility or inpatient rehab you get physical therapy, but it isn't round the clock. Virtually all hip fractures are allowed weight bearing as tolerated.

The odd case in the middle

A femoral neck fracture treated with a total hip is the same operation as an elective total hip. Those patients often wake up feeling better than they walked in. And they still don't go home the same day.

What his father actually needed wasn't clinical

Eddie made the observation that reframes the anecdote. Nothing the family provided in Little Rock was clinical care. Cooper's medical training mattered, but what it gave him was access to information — a phone line to the surgeon and the confidence to know what normal looked like. Everything else was advocacy, education, and logistics. Those are social needs, and they are the ones that determine whether a discharge home holds.

Advocacy

Someone who can reach the care team and ask a direct question without waiting for the next appointment.

Education

Knowing what to expect on day three, and what genuinely warrants a call rather than an emergency department visit.

Resources

The practical scaffolding — someone at the house, a ride, a plan for the bathroom and the stairs.

Give a patient advocacy, education, and resources, and you catch the thing going off the rails while it is still small and still controllable. That is the whole mechanism — recovery goes faster, quality goes up, and the episode stays inside its budget as a byproduct.

Eddie Qureshi, reframing the case

Which means the work extends past the system

Directing more hip fracture patients home requires the buy-in of families, not just clinicians. That is a conversation a health system has never had to be good at, and now has ninety days of financial reason to learn.

And a population that refuses to generalize

He is emphatic that hip fracture patients are heterogeneous. Some are barely older than he is. Some are very frail at 87. There will always be a need for post-acute inpatient care.

Medicare named the hospital. The hospital is not the whole story.

Asked why new care models draw resistance when they are process innovations rather than clinical ones, Cooper went straight back to his private practice years — because that is where the can of worms is. Medicare has been indiscriminate in this generation of models: every acute care hospital is a participant, whether its physicians are salaried, contracted through medical groups, or independent. From the ground up, those arrangements are not the same thing at all.

INTEGRATED

Easier to not be resistant

Inside a good-sized system, resources and effort can be pointed at the problem. It lets the surgeon operate and be a doctor without having to involve themselves in any of it. He guesses resistance is lower here for exactly that reason.

INDEPENDENT

A list of open questions

How will an independent hospital and an independent surgeon group cooperate? How does compensation flow down? With a bundled payment across ninety days, who bears the most risk — and who is the accountable entity?

He does not pretend to know the answers. He does not know whether it will be prescriptive — this percentage to the surgeon, this percentage to the facility — or left to the parties. Those questions may get answered. Right now they aren't.

The town-sized version of the problem

His example is Eddie's hometown — Batesville, Arkansas. A hospital and a small orthopedic group. Not tiny, not big by any stretch. His friends there used to be in private practice; now they're integrated, so they will naturally work together. There are towns like that all over the country, and he says honestly that he doesn't know what the answer is for them.

THE SIDE EDITORIAL

This is another of the myriad forces pushing physicians into integration with hospital systems. Not the only one — reimbursement has been doing it for years — but another push in the same direction.

Said by someone whose heart is still partly in private practice.

The post-acute partnership nobody has designed yet

Ninety days pulls skilled nursing facilities, outpatient therapy clinics, primary care, and community organizations into an episode the acute care hospital is financially accountable for — while the quality and satisfaction those partners deliver sit outside its walls. Cooper calls it a really hard problem, and does not soften it.

There's probably room for less use of post-acute facilities. There is no room for no use of them. And that facility is expecting reimbursement.

What he expects to happen

Post-acute facilities will be anxious to partner with hospitals. They will also have to take some marching orders from them. That is a different relationship than the referral pattern they have now.

Why owning one isn't the answer either

Mercy has an acute rehab facility in Northwest Arkansas, and it is capped on how many basic orthopedic patients it can take — it has to take stroke patients and others. Most hip fracture patients still go to independent skilled nursing facilities.

He notes it is structurally the same question as the one facing private practice groups and small independent hospitals — two parties, one episode, one payment, and no settled rule for how the money and the accountability split. His summary of the situation is four words long.

“A lot of challenges, and not a lot of time to meet them.”

Ninety days is a very long time to track by hand

Eddie asked whether technology is one of the tools that keeps clinicians clinical rather than administrative. Cooper upgraded the premise. At this point, he says, technology is more than an enabler — it is essential. He isn't sure this can be done well without it.

KEEP UP

With the data

Outcome measures collected across an extended window, for every patient, not a sample.

FIND

The efficiencies

You cannot act on a discharge pattern you cannot see while the episode is still open.

AVOID

Pen and paper

It was being done in a rudimentary way twenty years ago by Arkansas payers. It left a lot to be desired.

“People say ninety days isn't very long. It's very long. And I think it's essential.”

Dr. Scott Cooper, on technology in the episode

The distinction he draws is worth holding onto. The argument for technology here is not efficiency for its own sake, and it is not administrative relief — though he wants that too. It is that the measurement itself is now the product. A ninety-day episode judged on patient-reported outcomes, all-cause readmissions, and total cost cannot be run on recollection and a fax machine, and the organizations that treat reporting as a back-office task will find out the expensive way.

Fourteen months, and one honest piece of advice

TIME REMAINING

14

months until CJR-X goes live

3,300

acute care hospitals in scope

Asked what surgeons, clinicians, and administrators should do with the runway, his first answer is a joke that isn't one: start six months ago. The second is to be open to engaging with technology platforms that know this in and out.

A big system has people

He found out, from outside the loop these days, that Mercy is already looking hard at CJR-X. They know it's coming and they know what to do. That is the advantage of scale.

One size does not fit all

What works at Mercy is not what works in Batesville. Whatever you build has to accommodate that variability and still work for every party in the episode.

“There's not more money coming into the healthcare system. We have to find a way to do it well — do it even better than we've done it when it comes to the patients — but do it in a more efficient way.”

Dr. Scott Cooper

He is candid about what is driving the timeline. A nice talk about doing better for patients is real — that is the surgeon's concern and always has been. But the forcing function is economics. Medicare is beyond broke. He would rather the field name that plainly and get on with finding the efficiencies than pretend the deadline is optional.

What's best is often the least intuitive thing on offer

His optimism rests on a specific belief: there is room for improvement, and the improvements available are frequently the ones that look wrong on the surface. Less is often more, and patients do not arrive believing that. Two stories make the point better than an argument would.

THEN

The CPM machine

Continuous passive motion after knee replacement was abandoned a long time ago — demonstrated pretty conclusively not to improve outcomes. It was still a hard sell. One patient he's fond of had her first knee with a CPM and her second without, and years later still tells him the CPM knee is the better one.

NOW

The quiet knee

A newer rehab strategy with essentially no therapy for the first couple of weeks — treating a knee replacement like a bad ankle sprain. You don't beat up a sprain; you rest, ice, compress, elevate. He hasn't adopted it yet and is openly curious. One of his former partners had his own knee replaced and bought in completely.

A savings opportunity, incidentally

Skipping outpatient physical therapy for the first two weeks removes cost from the episode. It could also be a path to better outcomes. Both of those land inside a bundle.

And a harder sell than it sounds

It only works if you can convince patients that everything will be all right without therapy the day after surgery. That is the same conversation as sending a hip fracture home.

The pattern he has watched for thirty years

Hip and knee replacement patients used to spend four or five days in the hospital and then go somewhere else. The length of stay got whittled down year by year, and every step of it required convincing patients first. Getting fracture patients home earlier will be another of those transitions. Patients uniformly want to go home. It is the families who aren't so sure — for good reasons, because people work and somebody can't be with mom all the time.

Healthcare is being forced to innovate at an unprecedented pace — through the models, and through the technology. Both men said they remain optimistic about where it takes us.

The ninety days that happen outside your building

Rainfall Health is an accountability and accessibility platform powered by AI — the first and only company certified for these Medicare-mandated programs. The gaps Cooper describes are the ones it is built for: patients at home who need advocacy rather than clinical care, post-acute partners who don't share your chart, and an episode whose performance you need to see while it is still open.

Support that travels home

Around-the-clock patient and caregiver support with escalation pathways — the advocacy, education, and resources that make an earlier discharge hold.

Outcomes, captured continuously

Patient-reported outcome measures collected across the full 30- or 90-day window, for every patient, without a team calling each one by hand.

Built for the mandate

Quality-gate metrics, risk-adjusted episode performance, and the reporting TEAM and CJR-X require — as one system rather than a stack of vendors.

FROM THE CONVERSATION

“It's Rainfall — knows the landscape, knows what's going on, and has a vision for how this can be done.”

Dr. Scott Cooper, unprompted

The variability he warns about is the design constraint, not an objection. An operation that serves a 300-clinician integrated system and a two-surgeon group in a town of 10,000 has to be the same platform configured differently — which is the harder engineering problem, and the only one worth solving.

GET CONNECTED

Fourteen months is not a long runway

If you're preparing for TEAM or CJR-X — building the post-acute relationships, capturing patient-reported outcomes, or working out how a 90-day episode holds together across parties who don't share a chart — we'd like to help. Rainfall is the end-to-end solution, technology and services, for hospitals and health systems.

LEARN MORE

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START A CONVERSATION

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Watch for the next episode of the Rainfall Health Podcast.



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